



AMERICAN COLLEGE OF GASTROENTEROLOGY

11333 Woodglenn Drive, Suite 100, North Bethesda, Maryland 20852-3071
P: 301-263-9000; F: 301-263-9025; Website: gi.org

February 23, 2026

Mr. Robert F. Kennedy, Jr.
Secretary
Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Secretary Kennedy:

The American College of Gastroenterology (ACG) is pleased to respond to this *Request for Information (RFI): Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*. ACG appreciates the Trump Administration's request for public input to help define policy priorities and actions for the deployment and advancement of artificial intelligence (AI) in the healthcare sector. AI has already been integrated into many aspects of medical practice, and its role will continue to expand. A typical gastroenterology practice uses AI in numerous ways, including administrative tasks, diagnosis and treatment, patient communication, and interactions with payors. Given the rapid pace of innovation and a lack of consistent policy parameters, a federal framework to guide future AI-related actions is essential. Below we provide recommendations and strategies to advance the use of AI for clinical care.

ACG is a physician organization representing gastroenterologists and other gastrointestinal (GI) specialists. Founded in 1932 and representing over 20,000 GI clinicians, ACG's mission is to enhance our members' ability to provide world-class care to patients with digestive disorders and to advance the profession through excellence and innovation, grounded in the pillars of patient care, education, scientific investigation, advocacy, and practice management. ACG leaders have been actively engaged in developing AI for GI, working with subject-matter experts in our specialty to harness its benefits for patient care and address the challenges medical providers and practices face in achieving this goal.

We recommend addressing the following key barriers in the adoption of AI in the clinical environment:

Reimbursement

- ACG urges the agency to explore and model the implications of implementing revisions to the definition of “software” for the Medicare Physician Fee Schedule (PFS).

Regulations

- ACG supports the Administration’s efforts to reduce the regulatory burden on the development and deployment of AI in the healthcare sector. We urge the Administration to establish policies that truly reduce the burden and do not merely shift responsibility for safety and efficacy from the developer to the practicing clinician or other users of AI-enabled medical technology.
- The current regulatory framework for medical liability related to AI-enabled medical technology is inadequate and unclear. Policymakers must develop clearer liability rules and establish liability protections for the use of AI in healthcare.
- ACG recommends that the federal government incorporate proper medical oversight of AI-enabled technology into any regulatory framework advanced by the Administration.

Establishing a Reimbursement Pathway

The adoption of AI across the healthcare continuum raises several issues, including the development of a reimbursement pathway to ensure continued innovation. In gastroenterology, AI-enabled technology is used for endoscopy, image recognition, clinical data analysis, and practice management.

The healthcare community’s current challenge is the lack of a methodology to capture the costs of AI-enabled technologies that may enhance medical treatment. There is a lag between the development of AI-enabled technology and the establishment of reimbursement pathways to capture its costs. The absence of a clear reimbursement pathway discourages innovation and reduces developers’ interest, leaving patients unable to benefit from these advances. While there will not be a one-size-fits-all reimbursement solution for AI, reimbursement is essential to sustaining innovation in AI-enabled technologies and improving adoption of these technologies.

Stakeholders have proposed and CMS has, in limited cases, adopted a solution to establish a predictable reimbursement pathway for the use of AI-enabled technology in the Medicare outpatient setting: assigning a new AI-enabled medical device to a new technology ambulatory payment classification (APC). While the new technology APC is available, there are limitations in the utility, coupled with uncertainty and unpredictability that chills adoption. The types of AI-enabled technologies that receive new technology APCs are often expensive, and absent consistent reimbursement, providers face difficult

decisions regarding whether to invest in such technologies. ACG urges CMS to further explore this option and work with stakeholders to assess its feasibility and potential unintended consequences.

In contrast, establishing a predictable payment pathway for AI-enabled medical technology under the Medicare PFS presents unique challenges. CMS has acknowledged the limitations of the current Medicare PFS in setting rates for AI-enabled CPT codes. The agency has stated, “... AI applications are not well accounted for [in] our PE methodology ... [we] recognize that practitioners do incur resource costs for purchase and ongoing use of software.” Similar comments were also made in its 2024 Medicare PFS Final Regulations.

The traditional Resource-Based Relative Value Scale (RBRVS) was not designed for software-based, non-human-labor technologies, creating challenges in assigning appropriate values to AI-enabled medical devices. ACG calls on CMS to work with stakeholders to identify alternative, formal, and predictable reimbursement pathways for the Medicare PFS.

CMS has stated that a significant challenge in accounting for the costs of AI-enabled medical devices stems from its current practice expense (PE) formula for calculating PE RVUs. Under this formula, AI software (often referred to as “software as service” or SaaS) is treated as overhead rather than a direct PE expense. As overhead, it is calculated as part of the indirect PE portion of the PE RVU formula. In fact, the formula treats all software as overhead. This was appropriate when the formula was developed and based on how software was used at that time. The role of software in providing medical services has evolved, and the PE formula should reflect this shift.

CMS has created a workaround by comparing, or “crosswalking,” PE RVUs for AI-enabled medical devices to codes with similar values to the estimated costs of the AI software. These estimates are generally based on invoices submitted by stakeholders. In 2026, there are only three Category I codes with AI-enabled technology, and this solution becomes less feasible as the number of AI-enabled Category I codes grows.

One potential solution is to revise the current methodology for calculating PE RVUs. In the definition of software, an exception can be made to treat AI software as a supply rather than as overhead. As a supply, it would be treated as a direct expense and accounted for under the direct PE portion of the PE RVU formula. We believe this approach would more accurately capture the actual costs of AI software and better reflect how it is used in delivering medical services.

ACG urges the agency to explore and model the implications of revising the definition of software for the Medicare PFS to more appropriately capture AI-enabled technologies used in the delivery of healthcare services.

Balancing Appropriate Regulatory Oversight, While Supporting Innovation

The adoption of AI across the healthcare continuum raises several regulatory issues, which emphasizes the need for designing a regulatory framework that truly reduces burden and does not merely shift responsibility; ensuring transparency about responsibility and liability in the use of AI when errors occur; and establishing appropriate oversight by medical professionals in the deployment of AI.

Supporting True Burden Reduction

ACG applauds the Administration for prioritizing administrative burdens on healthcare providers. We support a regulatory framework for AI-enabled medical technology that balances the flexibility needed to support innovation with appropriate oversight of evolving medical technology. For example, the AI regulatory sandboxes announced by President Trump to accelerate development and minimize federal oversight have great potential to spur innovation. It is also important that this concept not merely shift the burden of oversight from the AI technology developer to the AI technology user, or compromise patient safety.

Appropriate oversight of AI-enabled medical technology, as with other medical technologies, is needed to ensure it is fair, valid, effective, and safe. Throughout the technology's life cycle—from conception to development to implementation to ongoing maintenance—there are critical roles for the entities involved in each stage (e.g., technology developers, distributors, users). As the regulatory framework is designed, the technology's entire life cycle and the entities engaged with it should be considered, and appropriate roles and responsibilities assigned for regulatory oversight.

ACG supports the Administration's efforts to reduce the regulatory burden on the development and deployment of AI in the healthcare sector. We urge the Administration to establish policies that truly reduce the burden and do not merely shift responsibility for safety and efficacy oversight from the developer to the practicing clinician or other users of AI-enabled medical technology.

Medical Liability

Like any medical technology, AI-enabled technology carries risks. Given the rapid pace of AI adoption in the medical field, there is a critical need for clarity and consistency in determining the responsible use of AI and medical liability when errors occur. The current regulatory framework for medical liability is inadequate. The development of AI-enabled technology introduces variables that may not apply in the current environment and raises new medical-legal issues. Neither patients nor clinicians will feel comfortable using this technology unless the framework for responsible-use and liability determination is transparent. Standards for safe use and medical liability must be part of any national strategy for AI.

The current regulatory framework for medical liability related to AI-enabled medical technology is inadequate and ill-suited to the challenges posed by AI. Policymakers must develop clearer liability rules for the use of AI in healthcare.

Engagement with Healthcare Professionals

AI-enabled technology by private payers is a growing concern for prior authorization and other claims-processing activities. AI-enabled prior authorization has the potential to significantly reduce time-consuming manual tasks for clinicians and payers. Algorithms can also be used to determine whether requested treatment and submitted justification documents comply with the plan's medical criteria. Algorithms can be used to triage prior authorization decision-making to an appropriate reviewer.

While these are appropriate uses for AI, ACG is concerned about using AI to make prior authorization decisions without human review. Trained medical professionals need to make these critical medical decisions, which can profoundly affect the trajectory of a patient's treatment and health outcomes. There have been real-world examples of payors making prior authorization decisions using AI-enabled technology without medical professionals' review and input. Some states have begun passing legislation to bar this practice. Federal leadership is needed to set standards for appropriate medical oversight of AI-enabled technology.

Thank you for the opportunity to submit comments on how the Department of Health and Human Services can support the rapid adoption and use of AI for clinical care. We hope to continue to engage with the Administration on this critical issue. Please contact Brad Conway, Vice President of Public Policy, Coverage & Reimbursement, at 301-263-9000 or bconway@gi.org with any questions.